Lakeshore Regional Medical Center

Department of Anatomical Pathology · SURGICAL PATHOLOGY REPORT

Patient: Karen Johnson SSN: 943-43-8311
DOB: 04/02/1959 MRN: MRN3057514
Phone: (927) 555-2063
Accession No: S25-10706 Collected: 10/20/2025 Reported: 10/29/2025
Requesting surgeon: Dr Mohd Abdullah

PATIENT HISTORY: The patient is a. A colonoscopy showed partially obstructing. fungating tumor of the left colon close to splenic flexure. No other abnormalities seen. Bowel prep was suboptimal as a result of. partial obstruction of poor bowel prep, evaluation Mas not continued. Biopsy of this lesion revealed adenocarcinoma. PRE OP DIAGNOSIS: Colon cancer. POST OP DIAGNOSIS: Same. PROCEDURE: Not given,. ADDENDA: Addendum. At the request of. EGFR immuhostaining was performed on a section of the colon cancer (slide 1H). The immunohistochemical stain for EGFR is negative, but this interpretation is based on an altered method (an original. H&E stained slide from this case had to be de-stained in order to be stained for EGFR, a process that could potentially. interfere. with. the. immunohistochemical reaction). Thus, an additional EGFR stain has been requested on this patient's. metastatic tumor. and is pendino: the results will be reported as an addendum of. FINAL DIAGNOSIS: PART 1. AND 2: LARGE INTESTINE, PARTIAL RESECT ON WITH SUBSEQUENT REVISION OF "NEW PROXIMAL MARGIN" -. A. INVASIVE MODERATELY DIFFERI INTIATED COLONIC ADENOCARCINOMA EXTENDING THROUGH THE. MUSCULARIS PROPRIA INTO THE SEROSAL ADIPOSE TISSUE. DIAMETER 6.5 CM. NO EVIDENCE OF. ANGIOLYMPHATIC INVASION. PERINEURAL INVASION IDENTIFIED. ALL MARGINS FREE. PATHOLOGIC. STAGE T3N0MX, DUKES' B, ASTI ER COLLER B2. B. TWELVE (12) PERICOLONIC LYMAH NODES WITH NO EVIDENCE OF METASTATIC CARCINOMA.

COMMENT: Patient's sister Patricia Thompson requested a copy; contact (927) 555-2063.

Electronically signed by Dr Wong Hui Min, Consultant Pathologist, on 10/29/2025

SYNTHETIC DEMO DOCUMENT - all identifiers are fake. Report text: TCGA-Reports (Kefeli & Tatonetti, 2024).